



QUICK SCREEN

MOVEMENT READINESS SCREEN

MANUAL

VERSION 1.0

The information contained herein is not intended to be a substitute for professional medical advice, diagnosis or treatment in any manner. Always seek the advice of your physician or other qualified health provider with any questions you may have regarding any medical condition or before engaging in any physical fitness plan. All rights reserved. Printed in the United States of America using recycled paper. No part of this manual may be reproduced or transmitted in any form whatsoever without written permission from the author or publisher, with the exception of the inclusion of brief quotations in articles or reviews.

Copyright 2021 Functional Movement Systems.

Table Of Contents

Preface	3
Introduction.....	4
Screening Guidelines	6
Scoring the Screen.....	7
Cervical Patterns.....	9
Toe Touch Pattern	12
Shoulder Mobility Pattern	15
Rotation Pattern	18
Balance Pattern	21
Squat Pattern	23
Spine Extension Clearing.....	26
Tips to Run the Quick Screen	27
The Quick Screen Score Sheet	28
Corrective Strategy Algorithm.....	30
Protect, Correct, and Develop	31
Corrective Strategy Example: Cervical Patterns.....	32
Corrective Strategy Example: Toe Touch	33
Corrective Strategy Example: Shoulder Mobility	34
Corrective Strategy Example: Squat.....	35
Corrective Strategy Example: Balance.....	36
Corrective Strategy Example: Rotation.....	37
Appendix 1 - Verbal Instructions	38
Appendix 2 - Modified FMS	41



Preface

Functional Movement Systems (FMS) was formed to improve how movement is evaluated across fitness, performance, and rehabilitation settings, with the goal of identifying movement dysfunction and improving communication between professionals. This work is based on the observation that fundamental movement patterns must be assessed before isolated joints or corrective strategies are considered, leading to the development of the Functional Movement Screen (FMS) as a proactive baseline for active populations and the Selective Functional Movement Assessment (SFMA) for individuals experiencing pain.

The Quick Screen was developed from this same foundational philosophy, with an emphasis on simplicity, efficiency, and early decision-making. It prioritizes pain first, followed by basic movement observation, allowing professionals to quickly determine whether an individual can safely proceed, requires modification, or needs further evaluation. Rather than replacing existing assessments, the Quick Screen serves as an entry point—providing a clear, consistent starting place for screening movement.

Since this system was first introduced, the evidence has continued to suggest that this fundamental philosophy should be followed in order to have the greatest impact for active individuals seeking to improve their level of fitness or patients looking to get back to function. FMS continues to analyze data, review research and gather feedback from professionals so that the evolution of our philosophy and message can continue. We will always work to the best of our ability to bring the latest and greatest information related to movement to the fitness, medical, and strength and conditioning professions so that we can enhance the lives of the individuals we touch.

Introduction

The Quick Screen presents a practical, common-sense approach to movement screening designed to provide a quick, accessible entry point for evaluating movement quality. Developed through decades of professional feedback and long-term data analysis, this screening process has been refined to focus on the most meaningful movement patterns while remaining efficient and easy to apply. While more detailed assessments may still be necessary for specific populations or clinical situations, this approach offers a streamlined way to gather valuable information that supports informed decision-making in rehabilitation, fitness, and wellness settings.

Movement screening is treated as a responsibility rather than a checklist, with emphasis placed on whether the process makes sense both at the initial assessment and during re-evaluation over time. Advances in data collection and analysis have made it possible to reduce complexity without sacrificing accuracy, allowing professionals to screen movement more quickly while maintaining scientific integrity. The result is a clear, structured process that flows smoothly, removes unnecessary barriers, and provides a foundation that can be expanded as deeply as needed based on experience and context.

OVERVIEW OF MOVEMENT PATTERNS SCREENED

This movement screen evaluates a series of fundamental patterns that collectively provide insight into an individual's movement quality, coordination, and readiness for daily activity and training. Each pattern was selected because it reflects how the body naturally moves, integrates multiple joints and systems, and manages mobility and stability under simple, controlled conditions. Rather than isolating single muscles or joints, the screen observes how the body organizes movement as a whole.

The screen begins by assessing the cervical spine to ensure the neck can move comfortably through basic ranges without pain, establishing a foundation for safe movement elsewhere in the body. From there, the screen evaluates upper-body patterns such as shoulder mobility, which reveal how the shoulders function together in reciprocal motion and how posture, asymmetry, and trunk control influence upper-quarter movement.

Lower-body and trunk patterns are assessed through movements such as the squat and toe touch, which examine sagittal-plane control, sequencing, and the integration of mobility and stability through the hips, spine, knees, and ankles. Rotational capacity is evaluated using a full-body rotation pattern that highlights side-to-side differences, weight-shifting strategies, and compensatory movement behaviors. Balance is assessed through single-leg stance patterns to examine stability, sensory integration, and proprioceptive control with and without visual input.

Together, these patterns provide a broad, yet efficient snapshot of how the body moves, compensates, and responds to simple movement challenges. The goal is not to diagnose or correct during the screen, but to identify pain, limitations, and asymmetries that inform safe, effective decision-making in fitness, rehabilitation, and wellness contexts.

BASIC MOVEMENT PRINCIPLES

Effective movement practice is guided by three foundational principles that prioritize quality, safety, and consistency. Together, they provide a clear framework for observing, screening, and developing movement in a responsible and sustainable way.

The first principle is to **move well before moving often**. Movement quality must meet a minimum standard before volume, intensity, or frequency are increased. Building capacity on poor movement is unsustainable and often leads to compensation or injury. Moving well allows the body to respond appropriately to its environment, while moving often creates the exposure needed for adaptation over time. Quality establishes the foundation; quantity supports long-term development.

The second principle is to **protect, correct, and then develop movement**. Protection comes first and may involve modifying activities, avoiding painful or dysfunctional patterns, or referring to a healthcare professional when appropriate. Correction focuses on improving movement quality once a safe baseline is established. Only after movement is protected and corrected should higher levels of load, volume, and performance be developed. This approach reinforces quality before capacity and places responsibility and safety at the forefront.

The third principle is to **create systems that support these decisions**. Standardized screening, clear criteria, and consistent procedures provide objective baselines for movement. Treating movement as a vital sign allows professionals to identify missing, limited, or dysfunctional patterns and make informed decisions. Systems that match risk to the desired outcome help ensure accountability, consistency, and long-term success.

Together, these principles ensure that movement decisions are intentional, ethical, and practical—supporting sustainable progress rather than short-term gains built on compromised movement quality.

PAIN AND MOVEMENT SCREENING

Pain is the most important variable identified during a movement screen and must be addressed before focusing on performance, mobility, or corrective strategies. The primary purpose of screening is not to improve movement immediately, but to first determine whether movement can be performed without discomfort. Pain fundamentally changes how the body moves and how movement patterns should be interpreted.

The presence of pain during simple, low-stress movements such as reaching, rotating, squatting, or touching the toes is increasingly common across general populations. This trend highlights the need to take pain seriously rather than normalize it or work around it without acknowledgment. Pain should not be viewed as something to tolerate, mask, or push through, but as meaningful information indicating that the body is experiencing disharmony.

Individuals respond to pain in different ways, often adopting compensatory movement strategies to protect sensitive areas. These adaptations can significantly alter movement quality and create inconsistencies across patterns, which are valuable observations during screening. For this reason, pain is recorded separately from movement scores and treated as a priority finding.

When pain is identified during the movement screen, appropriate referral should be considered. **The screen is not intended to diagnose or treat pain, but to recognize its presence and guide responsible next steps.** Pain may indicate an underlying issue that requires evaluation by a qualified healthcare professional.

Referring individuals for further assessment helps ensure movement decisions are safe and informed. This reinforces professional boundaries and emphasizes that pain is a signal to be addressed, not ignored or trained through.

Screening Guidelines

This section outlines key testing considerations that support clear observation, consistent execution, and reliable interpretation of movement screens, helping professionals gather accurate and actionable information.

Footwear:

Clients should wear the shoes they train in most frequently to promote consistent and reliable screening conditions. Maintaining the same footwear across the initial screen and any subsequent re-screens helps ensure that changes in movement are due to actual adaptation rather than altered testing variables. Because most individuals spend the majority of their daily and training activities in shoes, screening in typical footwear provides a more accurate reflection of how they move within their current lifestyle.

No Warm-up:

The Quick Screen is performed without any prior warm-up, stretching, or movement preparation. It is important to know what a person's natural state of movement is when they walk through the door. This is the best indication of the quality and level of movement competency they experience in their current daily activity.

Positioning

Two things to consider when observing the movements of the screen are distance and movement. Considering these two things will take care of most of the issues involved in trying to see everything during the screen. Step back from the client to create enough distance, allowing you to see the whole picture at once. Most of the confusion over where to stand comes from being too close and too focused on one area of the test. Stand far enough away to allow a more global focus. View the entire movement and let the test criteria become evident.

Verbal Instructions:

This manual provides you with verbal instructions that should be stated to the client as specifically and consistently as possible. The verbal instructions are designed to guide the client you are screening into the proper set-up position and instruct them how to execute the movement. These instructions are designed to give them just enough information to clearly understand how to perform the test without adding coaching or feedback that would alter their natural movement pattern. This will ensure that you don't miss anything in the set-up as the screener and to also create consistency for each screen that you administer.

Screening Order:

The recommended order for screening takes the client from standing positions to ground-based positions, which is a physically efficient and time efficient process for the client while transitioning from one test to another. Although this is the recommended order of tests, the order the clients move through stations can start and end in any sequence and will not negatively affect the scoring results.

1. Cervical Flexion Clearing
2. Cervical Rotation and Extension Clearing
3. Toe Touch
4. Shoulder Mobility
5. Rotation
6. Balance
7. Squat
8. Spine Extension Clearing

Scoring the Screen

The client has three attempts to perform each test, so don't be afraid to move around during the test. Depending on the test, standing to the side or facing the person may provide the best vantage point. Take advantage of all three trials and move around if the score is not obvious from one point of view.

Movement Pattern Scoring

Movement patterns are scored on a scale from 0 to 3, which represents four distinct categories of movement quality. While a numerical value is assigned, the score itself reflects a category rather than a judgment of performance quality in absolute terms.

A **score of 3** indicates the individual performs the movement pattern exactly as directed, demonstrating optimal movement. This does not imply perfection, but rather that the movement meets all intended criteria.

A **score of 2** is given when the movement is completed but with noticeable compensation. This is common and considered acceptable. At a minimum, individuals should be able to achieve a score of 2 on each movement pattern.

A **score of 1** indicates the individual is either unable to achieve the required position or unable to perform the movement as instructed, reflecting significant compensation and unacceptable movement quality.

A **score of 0** is assigned when pain is present at any point during the movement, regardless of how well the movement is performed. Pain may occur anywhere in the body and at any phase of the pattern. Importantly, pain overrides all other scoring considerations—if pain is reported or observed, the final score for that movement is always a zero, even if the movement otherwise appears optimal.

Scoring Movement Patterns			
3	2	1	0
Optimal Movement	Acceptable Movement	Unacceptable Movement	Pain Present
Pattern performed as directed	Pattern performed with compensation	Unable to perform pattern correctly	Pain occurs during movement pattern
! When Pain Is Present, Score Is 0			

Attempt Guidelines & Scoring Rules



Up to 3 Attempts

Maximum of three attempts per movement pattern.

Record the Best Score

Scoring Example



Attempt 1

1



Attempt 2

2



Attempt 3

2



Final Score

2



When in Doubt, Score Low

If unsure, choose the lower score.

Each movement pattern is performed for up to three attempts. Research and feedback indicate that allowing more than three attempts does not typically improve performance, and the goal of the screen is to remain efficient and time-effective. Verbal instructions may be repeated between attempts as needed.

The best performance out of the three attempts is recorded as the final score. For example, if a participant scores a 2 on the first attempt and a 3 on the second, the final score is a 3. If the participant performs the movement optimally on the first attempt, no additional repetitions are necessary.

When uncertainty exists, scorers should default to the lower score. If there is any doubt or lack of confidence that the individual fully meets the criteria for a higher score, the lower score should be assigned to ensure consistency and accuracy in scoring.

If a score of 3 is achieved, the test is complete and no further attempts are necessary, as a higher score is not possible. If pain is reported at any point during the test, the score is recorded as 0.

Clearing Test Scoring

Clearing tests function primarily as pain provocation tests, using positive/negative criteria rather than the 0–3 movement scale. Pain is documented as positive, while the absence of pain is documented as negative. If there is any uncertainty about whether pain is present, the test should be scored as positive to remain conservative.

Any positive clearing test automatically results in a score of zero for the associated movement pattern, regardless of performance quality. Pain always overrides movement scoring.

Additionally, for cervical patterns, range of motion is assessed using pass/fail criteria. A pass indicates the required range of motion is achieved, while a fail indicates insufficient movement.

Scoring the Clearing Tests

Clearing tests function primarily as **pain provocation tests**, using **positive/negative** criteria rather than the 0–3 movement scale.

**Positive**
If pain is present

**Negative**
If no pain is present

When unsure, score **positive** to be conservative.

 **Any positive clearing test results in a SCORE OF ZERO** for that movement pattern.

For cervical patterns, range of motion is assessed with **pass/fail** criteria:

**PASS**
Required **Range of Motion** Achieved

**FAIL**
Insufficient **Range of Motion**

STARTING THE SCREEN

Before beginning the screen, the following statement should be clearly communicated to the client. These instructions apply to all tests and establish expectations for safety, understanding, and movement execution. Clear communication at the start helps ensure accurate scoring and allows the client to participate confidently throughout the screening process.

The following statement begins the screen and applies to all tests.

“Please let me know if there is any pain during the following movements. And if at any time you do not understand the instructions, stop me for clarification. We are going to perform each movement one at a time and in a smooth and controlled motion. Please wait for me to confirm that you have the proper set-up position and then I will signal you to begin the movement.”

Cervical Patterns

The Cervical Spine Tests are used to identify significant movement restrictions or pain in the neck that may influence overall movement quality and training readiness. As with all screening procedures, the focus is not on diagnosing specific conditions, but on determining whether cervical movements can be performed comfortably and without symptoms. The screen assesses the fundamental motions of the neck—flexion, extension, and rotation.

Because the cervical spine is integrally connected to the rest of the spine, dysfunction in the neck often reflects broader movement or postural issues rather than an isolated local problem. If cervical limitations are missed during screening, individuals may continue training or daily activities until symptoms escalate into more significant issues. For this reason, the Cervical Spine Screen is treated as a pass-fail assessment focused on identifying major concerns early. Establishing cervical movement tolerance first ensures that subsequent movement screens and training decisions are made with appropriate awareness and safety in mind.

CERVICAL FLEXION

Cervical flexion is assessed to determine the individual's ability to comfortably move the neck into a forward-bending position without pain or restriction. This movement provides insight into the mobility of the cervical spine as well as the individual's tolerance to flexion-based loading and positioning. During the screen, attention is placed on smooth, controlled motion and the presence or absence of discomfort rather than the exact degree of range achieved. Difficulty performing cervical flexion or the onset of pain may indicate underlying movement restrictions, postural influences, or neuromuscular guarding.

HOW TO DO THE CERVICAL FLEXION TEST

The Cervical Flexion Test is performed with the individual standing upright, feet together, and the arms relaxed at the sides. The individual places two fingers stacked vertically at the top of the sternum as a reference point. With the mouth remaining closed, the individual slowly lowers the chin toward the fingers, moving through cervical flexion under control. The end position is briefly held for a one-count before the individual returns to the starting position.

CERVICAL FLEXION VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Stand tall with your feet together, arms at your sides, and mouth closed.**
- **Place two fingers stacked one on top of the other, at the top of your chest bone.**
- **While keeping your mouth closed, slowly bring your chin down to your fingers.**
- **Hold the bottom position for a count of one and then return to the starting position.**

SCORING THE CERVICAL FLEXION TEST

There are two parts to the scoring of the Cervical Flexion Test. Firstly, determining if the range of motion is a PASS or FAIL based on if the individual can flex their neck to within two fingers from sternum. Secondly, is the movement POSITIVE or NEGATIVE for pain.



PASS = CAN flex neck within two fingers of sternum



FAIL = CANNOT flex neck within two fingers of sternum

POSITIVE = HAS pain

NEGATIVE = NO pain

CERVICAL ROTATION AND EXTENSION

Cervical rotation is assessed to evaluate the individual's ability to turn the head comfortably to each side without pain or restriction. This movement provides insight into neck mobility, symmetry, and tolerance to rotational motion that is commonly used in daily activities. During the screen, emphasis is placed on smooth, controlled movement rather than forcing end range. Limitations in cervical rotation or the presence of pain may indicate postural influences, joint restrictions, or neuromuscular guarding. Observing side-to-side differences also helps identify asymmetries that may influence overall movement quality and should be considered before progressing to more demanding movement patterns.

Cervical extension is screened to determine whether the neck can move into backward bending without pain. The focus is straightforward: can the movement be performed without causing pain. Extension is often assessed in combination with rotation to challenge the cervical spine in an end-range, closed position where major issues are more likely to appear.

Observing side-to-side differences also helps identify asymmetries that may influence overall movement quality and should be considered before progressing to more demanding movement patterns.

HOW TO DO THE CERVICAL ROTATION AND EXTENSION TEST

The cervical rotation and extension test is performed with the individual standing upright, feet together, arms relaxed at the sides, and the mouth remaining closed throughout the movement. From this position, the individual slowly rotates the head to one side as far as possible under control and holds the end position briefly for a one-count. Without returning to neutral, the individual then lowers the ear toward the same-side shoulder, combining rotation with extension and side bending. The individual returns to the starting position and repeats the sequence on the opposite side.

CERVICAL ROTATION AND EXTENSION VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- Stand tall with your feet together, arms at your sides, and mouth closed.
- While keeping your mouth closed, slowly rotate your head to the right as far as possible.
- Hold the position for a count of one.
- Now lower your right ear towards your right shoulder.
- Return to the start position.
- Repeat the same process by turning your head to the left as far as possible, then lowering your ear towards your left shoulder.

SCORING THE CERVICAL ROTATION AND EXTENSION TEST

There are two parts to the scoring of the Cervical Rotation and Extension Test. For Cervical Rotation, first determine if the range of motion is a PASS or FAIL based on if the individual can rotate their neck so their chin is at least to the halfway point of their clavicle. Next, is the movement POSITIVE or NEGATIVE for pain.



PASS = CAN rotate neck so chin is to at least halfway point of clavicle



FAIL = CANNOT rotate neck so chin is to at least halfway point of clavicle

POSITIVE = HAS pain

NEGATIVE = NO pain

For Cervical Rotation with Extension, it scored as a POSITIVE or NEGATIVE for pain.



POSITIVE = HAS pain

NEGATIVE = NO pain

Toe Touch Pattern

WHY THE FORWARD BENDING PATTERN?

The Toe Touch test is a fundamental sagittal-plane movement screen used to assess how the body coordinates mobility and stability during forward bending. Although commonly viewed as a simple test of hamstring flexibility, the toe touch provides much deeper insight into movement quality, sequencing, and motor control. A clean, efficient toe touch follows a predictable pattern in which the hips initiate the movement first, allowing the spine to flex smoothly afterward. This hips-first sequence reflects proper timing, coordination, and control throughout the kinetic chain.

When this sequence breaks down—most often when the head and spine move first—the limitation is rarely only a mobility issue. Instead, it also indicates a stability and sequencing problem. In this pattern, the body's center of gravity shifts forward prematurely, and the hamstrings are forced to act as a braking mechanism to prevent loss of balance. Over time, this compensation can be misinterpreted as "tight hamstrings," even though the underlying issue is inefficient movement strategy rather than tissue restriction.

The Toe Touch test is not about diagnosing why the movement is limited, but about observing whether the individual can perform the movement, how it is performed, and whether it is pain-free. By focusing on sequencing and control rather than simply range of motion, the Toe Touch test serves as a valuable screen for identifying movement inefficiencies influenced by prior injury, postural habits, or altered motor control.

WHY TEST IN THE STAGGERED STANCE?

A staggered stance is used to reduce compensation and reveal true movement patterns by narrowing the body's base of support and minimizing the ability to rely on symmetrical stability. In a parallel stance, individuals can often mask deficits by shifting weight, locking joints, or using bilateral support to maintain balance. The staggered stance challenges these strategies by placing the body in a more asymmetrical position, which requires improved coordination, timing, and control.

By offsetting the feet, the staggered stance helps limit excessive contribution from the ankles and knees and encourages the hips and trunk to manage stability more effectively. This positioning also brings subtle balance and rotational demands into the movement, making it easier to observe how the body organizes itself under less stable conditions. As a result, sequencing errors, stability limitations, and side-to-side differences become more apparent.

Ultimately, the staggered stance is not used to make movements harder, but to make movement quality clearer. It allows professionals to better assess how well an individual can control posture, load, and alignment when symmetry and passive stability are reduced, providing more accurate insight into underlying movement capacity.

HOW TO DO THE TEST

The toe touch begins from an upright standing position with the feet initially together and the arms relaxed at the sides. One foot steps forward so the inside of the forward heel aligns with the end of the opposite big toe, creating a narrow, staggered base of support. The hands are then placed on top of each other in front of their body. With the knees remaining straight but not locked, the individual initiates a forward bend. The goal is to reach toward the toes of the back foot while maintaining balance and control. After completing the movement, the individual returns to an upright posture, maintaining the same stance throughout the test. They are given three attempts to touch the toes of the back foot, while meeting all the criteria.

If the individual is unable to touch the toes of the back foot after three attempts, the individual again bends forward with the knees kept straight but not locked, this time reaching toward the toes of the front foot before returning to an upright position. They are given three attempts to touch the toes of the front foot, while meeting all the criteria.

The foot positions are switched so the opposite leg is forward, and the same sequence is repeated on the other side. This progression allows observation of side-to-side differences and further clarifies how the body manages balance, sequencing, and control in an asymmetrical stance.

We are naming this based on the back leg. If any of the criteria for a score of three are not achieved, the client receives a score of two. If any of the criteria for the score of two are not achieved, score this a one.

TOE TOUCH VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Start by standing with your feet together and your hands by your sides.**
- **Step forward with your right foot so that the inside of your right heel is even with end of your left big toe.**
- **Now, place your hands on top of each other.**
- **Keeping your knees straight but not locked out, look down and bend forward to try to touch the toes of your left foot.**
- **Return to the upright position.**

If after 3 attempts they are NOT able to touch the toes of their left foot, continue to the next part of the test:

- **Keeping your knees straight but not locked out, look down and bend forward to try to touch the toes of your right foot.**
- **Return to the upright position.**

Scoring the Toe Touch

The test is named for the leg that is in the back position. Each side should be scored accordingly.

3

- Ability to touch the toes of the back foot
- No change in starting knee position



2

- Inability to touch the toes of the back foot
- Able to touch the toes of the front foot
- No change in starting knee position



1

- Inability to touch the toes of the back or front foot
- Change in starting knee position



An individual receives a score of zero if pain is associated with any portion of this test.

Shoulder Mobility Pattern

WHY THE RECIPROCAL UPPER BODY PATTERN?

The Shoulder Mobility Test is a key movement screen used to assess upper-quarter mobility while simultaneously revealing postural control, coordination, and side-to-side asymmetries. While often assumed to evaluate the shoulder joint in isolation, this test provides a more global view of how the shoulders function together in reciprocal motion. Everyday and athletic movements such as climbing, swimming, running, throwing, and punching all rely on one arm moving overhead while the opposite arm moves behind the body, using each side as a counterbalance for the other.

Rather than relying on static postural assessments, the Shoulder Mobility Test evaluates dynamic movement patterns. By placing the arms in opposing positions—one reaching overhead and the other reaching behind the back—the test exposes how the cervical spine, thoracic spine, scapulae, and shoulders coordinate through a natural rhythm of motion. Limitations or asymmetries that may not appear when assessing one shoulder alone often become apparent when both shoulders are required to work together.

Because most individuals develop asymmetries from handedness, occupational demands, sports participation, or prior injury, the Shoulder Mobility Test is particularly effective at identifying functional movement problems that have been reinforced over time. Significant differences between sides may indicate restricted mobility, altered stability, or compensatory strategies elsewhere in the upper body. Ultimately, this test helps determine how well the shoulders share mobility and stability, offering valuable insight into movement quality rather than isolated joint range of motion alone.

HOW TO DO THE TEST

Hand length is first measured using the client's dominant hand, from the wrist crease closest to the hand to the tip of the middle finger, and recorded for scoring reference. The individual then stands upright with the feet together and the arms relaxed at the sides. With both hands formed into fists and the fingers wrapped around the thumbs, the individual simultaneously reaches one fist overhead and down the back while the opposite fist reaches upward from below the back in a single, continuous motion. The reach is performed without adjusting or "creeping" the hands closer together after the initial placement. The distance between the fists is observed and recorded. The test is then repeated on the opposite side, with the other arm reaching overhead while the opposite arm reaches up the back, allowing comparison of shoulder mobility and side-to-side symmetry.

SHOULDER MOBILITY VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Stand tall with your feet together and arms hanging comfortably.**
- **Make a fist so your fingers are around your thumbs.**
- **In one motion, reach the right fist over the head and down your back as far as possible while simultaneously reaching your left fist up your back as far as possible.**
- **Do not "creep" your hands closer after the initial placement.**
- **Repeat with the left fist reaching over the head while the right fist reaches up your back.**

Scoring the Shoulder Mobility

The test is named for the arm moving overhead. Each side should be scored accordingly.

Hand Measurement:



Measure the client's dominant hand from the wrist crease closest to the hand to the end of the middle finger.

You will need a ruler, a dowel, or piece of paper to measure their hand length.

3

- Fists are within one hand length



2

- Fists are within one and a half hand lengths



1

- Fists are not within one and a half hand lengths



An individual receives a score of zero if pain is associated with any portion of this test.

Shoulder Clearing

PURPOSE OF THE CLEARING TESTS

Clearing tests are included to determine whether shoulder pain is present and to ensure that movement limitations are not being influenced by discomfort or underlying pathology. While the Shoulder Mobility Test primarily evaluates movement quality, coordination, and asymmetry, the presence of pain changes the meaning of the results and must be identified separately. Pain can alter motor control, limit range of motion, and cause compensatory strategies that mimic mobility or stability deficits.

By performing clearing tests, professionals can distinguish between a functional movement limitation and a potentially more serious issue that may require further evaluation. These tests help confirm whether the individual can move through shoulder positions without discomfort and provide important context for interpreting the screening outcome. If pain is reported during a clearing test, the focus shifts from movement optimization to injury awareness and appropriate referral. Clearing tests therefore serve as a critical safety and decision-making step, ensuring that screening results are interpreted accurately and responsibly.

HOW TO DO THE CLEARING TEST

The clearing tests begin by standing upright with feet together and the arms relaxed at the side. The individual then reaches one arm overhead, bending the elbow to touch the top portion of the opposite shoulder blade. Return to the start position, and then move the arm behind the back, reaching upward to attempt to touch the lower portion of the opposite shoulder blade.

You do not score this but instead watch for a pain response. If pain is produced, a positive (+) is recorded on the score sheet, and a score of zero is given to the entire shoulder mobility test.

Both movement patterns are then repeated on the opposite side, allowing assessment of pain response in each shoulder during overhead and behind-the-back positions.

SHOULDER CLEARING VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **While standing tall, reach your right arm over your head to touch the top of your left shoulder blade.**
- **Return to the start position.**
- **Now, reach your right hand behind your back to try to touch the bottom of your left shoulder blade.**
- **Repeat both patterns with your left arm.**



POSITIVE = HAS pain

NEGATIVE = NO pain

Rotation Pattern

WHY THE FULL-BODY ROTATION PATTERN?

The Rotation Test is included to screen total-body rotational movement and identify side-to-side asymmetries that are often overlooked in more linear movement assessments. Rotation is a complex, integrated pattern that involves coordinated contributions from the feet, hips, pelvis, trunk, rib cage, shoulders, cervical, and balance system. Because rotational movements are frequently compensated for, limitations may not be immediately apparent unless rotation is observed directly.

Although rotation is commonly assumed to be a mobility issue—often attributed to restrictions in the rib cage, spine, or hips—the test also reveals important stability and weight-shifting behaviors. Many individuals demonstrate asymmetrical rotation due to habitual loading patterns, such as consistently favoring one side during standing, work, or sport. These weight-shifting preferences can significantly influence rotational capacity even when joint range of motion appears adequate.

Rather than attempting to determine the source of a limitation during the screen, the purpose of the Rotation Test is to observe and score the movement as it presents. Subsequent screens for balance, shoulder mobility, and other movement patterns provide additional context that helps clarify whether poor rotation is driven primarily by mobility restrictions, stability deficits, or a combination of both. By assessing rotation as a full-body movement, this test helps expose major movement flaws while acknowledging the body's ability to mask deficits through compensation.

WHY TEST IN THE STAGGERED STANCE FIRST?

A staggered stance is used during the Rotation Test to reduce compensatory strategies and more clearly expose how the body manages rotational movement. When standing with the feet parallel, individuals can easily shift weight, rotate through the feet, or rely on symmetrical support to mask limitations. The staggered stance narrows the base of support and introduces asymmetry, requiring the individual to demonstrate more intentional control of weight shift, balance, and trunk rotation. This position helps limit excessive contribution from the ankles and feet, encouraging the hips, pelvis, and trunk to drive the movement. As a result, side-to-side differences, stability deficits, and inefficient sequencing become more visible, allowing for a more accurate assessment of true rotational capacity and control.

HOW TO DO THE TEST

The Rotation Test begins from an upright standing position with the feet together and the arms relaxed at the sides. One foot steps forward into a staggered stance so that the inside of the forward heel aligns with the end of the opposite big toe. The arms are then extended straight out in front of the chest with the fingers interlocked, thumbs pointing upward, and the hands aligned with their nose. Without changing foot position or base of support, the individual rotates the body toward the side of the forward foot as far as possible under control, then returns to the starting position.

If the individual is unable to rotate beyond 90 degrees after three attempts in the staggered stance, the assessment continues with the next variation. The individual returns the forward foot to meet the back foot, standing with the feet together while maintaining the arms extended straight in front of the chest with the fingers interlocked, thumbs up, and hands aligned with the midline. Without altering foot position, the individual again rotates the body to the right as far as possible under control, then returns to the starting position.

ROTATION VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Start by standing with your feet together and your hands by your sides.**
- **Step forward with your right foot so that the inside of your right heel is even with the end of your left big toe.**
- **Extend your arms directly in front of your chest with your fingers interlocked and thumbs up and in line with your nose.**
- **Without moving your foot position, rotate your body to the right as far as possible.**
- **Return to the start position.**

If after 3 attempts they are NOT able to rotate beyond 90 degrees, continue to the next part of the test.

- **Move your foot back so that your feet are together, now with your arms directly in front of your chest with your fingers interlocked and thumbs up and in line with your nose.**
- **Without moving your foot position, rotate your body to the right as far as possible.**
- **Return to the start position.**

Scoring the Rotation Test

The test is named for the direction the individual is rotating their body. Each side should be scored accordingly.

3

- Able to rotate past 90 degrees in a staggered stance
- No change in foot position



2

- Able to rotate past 90 degrees in a symmetrical stance but not in a staggered stance
- No change in foot position



1

- Unable to rotate past 90 degrees in a symmetrical stance
- Foot position changes



An individual receives a score of zero if pain is associated with any portion of this test.

Balance Pattern

WHY THE SINGLE-LEG STANCE PATTERN?

The Balance Pattern is used to assess single-leg stability and the body's ability to maintain balance under varying sensory conditions. By observing balance performance on both sides, the screen helps identify asymmetries and differences in movement control that may not be apparent during bilateral or visually guided tasks. These observations provide insight into how effectively an individual manages posture, alignment, and stability in a single-leg stance.

A key component of this assessment is proprioception, or the body's ability to sense position, movement, and balance without relying solely on vision. When visual input is reduced, the nervous system must rely more heavily on sensory feedback from the feet, ankles, and lower extremities to maintain stability. While some decline in balance is expected when the eyes are closed, a marked inability to maintain balance often reflects incomplete proprioceptive integration, which can persist following previous injuries—even when strength, range of motion, and basic movement abilities appear restored.

This distinction is especially valuable when reviewing injury history, as residual sensory deficits are not always evident through traditional strength or mobility testing. The Balance Pattern helps uncover these subtle limitations and provides an opportunity to educate individuals on how prior injuries can influence balance and movement confidence. By identifying side-to-side differences and sensory reliance, the test supports more informed movement decisions and targeted training strategies.

HOW TO DO THE BALANCE TEST

The Balance Test begins with the individual standing upright with the feet together and the arms relaxed at the sides. The individual lifts one knee and thigh until the thigh is approximately level with the waist, establishing a single-leg stance. This position is held with the eyes open while balance and postural control are observed.

If the individual is able to maintain balance for 10 seconds with the eyes open, the test progresses to the next phase. Without changing position, the individual then closes the eyes and attempts to maintain the same single-leg stance, allowing assessment of balance control when visual input is removed.

BALANCE VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Start by standing with your feet together and your hands by your sides.**
- **Lift your knee and thigh up to be in line with your waist.**
- **Hold this position, keeping your eyes open.**

If they ARE able to maintain balance for 10 seconds with eyes open, proceed to the next part of the test.

- **Now close your eyes and maintain this position.**

Scoring the Balance Test

The test is named for the leg the person is standing on. Each side should be scored accordingly.

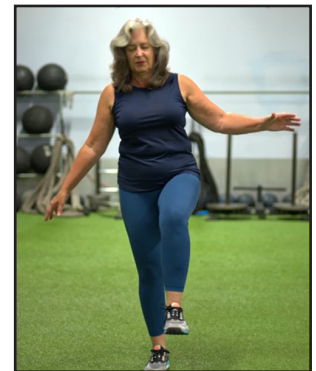
3

- Maintain single-leg balance for 10 seconds eyes open
- Maintain single-leg balance for 10 seconds eyes closed
- No significant sway
- No significant hip/thigh movement (lowering thigh)



2

- Unable to maintain single-leg balance for 10 seconds eyes closed
- Able to maintain single-leg balance for 10 seconds eyes open
- No significant sway with eyes open
- No significant hip/thigh movement (lowering thigh) with eyes open



1

- Unable to maintain single-leg balance with eyes open for 10 seconds
- Significant sway and/or hip/thigh movement(lowering thigh)



An individual receives a score of zero if pain is associated with any portion of this test.

Squat Pattern

WHY THE ARMS DOWN SQUAT PATTERN?

The Squat Test is a fundamental movement screen used to assess lower-body mobility, stability, and overall movement readiness for daily life. Although squatting was once a common human movement, modern lifestyles have significantly reduced how often—and how well—people perform it. As a result, the ability to squat efficiently can no longer be assumed and must be intentionally observed.

While the overhead squat is commonly used in athletic screening, this assessment utilizes a simplified squat pattern to reduce complexity and better reflect everyday movement demands. By avoiding overly technical upper-body positions, the test allows clearer observation of lower-body mechanics and trunk control. Shoulder mobility has already been evaluated elsewhere in the screening process, allowing this test to focus on the hips, knees, ankles, and core.

The purpose of the Squat Test is not to measure athletic performance, but to determine whether an individual can descend into a deep squat with control and without pain. Successfully achieving depth demonstrates integrated mobility and stability throughout the lower body. Difficulty reaching depth or the presence of pain may indicate a loss of an essential movement pattern that is critical for long-term function and movement efficiency.

WHY DO WE TEST WITH FEET TOGETHER?

Using a feet-together position during the Squat Test helps reduce compensatory strategies and reveals true lower-body movement capacity. When the feet are placed wider apart, individuals can often mask limitations by externally rotating the hips, shifting weight unevenly, or using a broader base of support to maintain balance. Bringing the feet together narrows the base of support and limits these compensations, requiring greater coordination, control, and symmetry through the ankles, knees, hips, and trunk. This position makes restrictions in mobility, stability, or sequencing more apparent, allowing for a clearer assessment of the fundamental squat pattern and how efficiently the body can manage depth without relying on altered stance or positioning.

HOW TO DO THE SQUAT TEST

The Squat Test begins with the individual standing upright with the feet together and the arms extended straight out in front of the chest, hands held in a fist position. From this position, the individual descends into a squat as deeply as possible while keeping the heels in contact with the ground and the chest upright. At the bottom of the squat, the fists are lowered toward the floor alongside the feet in an attempt to make contact with the ground. The individual then returns to the starting position under control.

If the individual is unable to place the fists on the ground at the bottom of the squat, the test progresses to a second variation. The individual again begins standing with the feet together, this time extending the arms in front of the chest with the thumbs pointing upward and the fingers fully extended. The squat is repeated using the same criteria for depth, heel contact, and upright posture. At the bottom position, the fingers are lowered toward the floor alongside the feet in an attempt to make contact with the ground before returning to the starting position.

SQUAT VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Start by standing with your feet together and extend your arms directly in front of your chest with your hands in a fist position.**
- **Now, squat down as far as you can, keeping your heels on the ground and chest tall.**
- **At the bottom squat position, lower your fists to your sides, trying to touch them on the ground alongside your feet.**
- **Then come back up to the starting position.**

If they are NOT able to put their fist to the ground, continue to the next part of the test.

- **Start by standing with your feet together and extend your arms directly in front of your chest with your thumbs pointing up and fingers straight out.**
- **Now, squat down as far as you can, keeping your heels on the ground and chest tall.**
- **At the bottom squat position, lower your fingers to your sides, trying to touch them on the ground alongside your feet.**
- **Then come back up to the starting position.**

SCORING THE SQUAT

3

- Able to touch fists to the ground beside feet
- Femur/thigh below horizontal



2

- Unable to touch fist to ground beside feet and/or unable to get femur/thigh below horizontal
- Able to touch fingers to the ground beside feet with femur/thigh below horizontal



1

- Unable to touch fingers to the ground and/or unable to get femur/thigh below horizontal



An individual receives a score of zero if pain is associated with any portion of this test.

Spine Extension Clearing

PURPOSE OF THE CLEARING TEST

The Spine Clearing Test is used to determine whether spinal extension can be performed comfortably and without pain, providing important context for interpreting movement screening results. Although discomfort during spinal extension is often assumed to indicate a structural back issue, pain during this movement is frequently influenced by asymmetries elsewhere in the body, particularly through the hips. Because the hip flexors connect the lower extremities to the spine, side-to-side differences can alter spinal mechanics and create uneven loading during extension, sometimes producing rotational stress or localized discomfort.

This test is not intended to diagnose spinal pathology, but to identify whether spinal movements are tolerated and to flag situations that may require further attention. A painful response during the Spine Extension Clearing Test may reflect unresolved movement asymmetries already identified in the screen, and improvements are often observed when those asymmetries are addressed and the test is rechecked. However, persistent or significant pain during spinal movements may indicate an underlying issue that should be evaluated further by a healthcare professional.

HOW TO DO THE CLEARING TEST

The Spine Clearing Test is performed with the individual lying prone on the floor, positioned with the hands placed directly underneath the shoulders and the palms resting flat on the ground. From this position, the individual presses the upper body upward by extending the elbows until the arms are fully straight, while the pelvis, hips, and lower extremities remain relaxed. The movement is performed under control, and the individual then returns to the starting prone position.

You do not score this but instead watch for a pain response. If pain is produced, a positive (+) is recorded on the score sheet.

SPINE EXTENSION CLEARING VERBAL INSTRUCTIONS

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

- **Lie on your stomach, with your hands underneath your shoulders and your palms facing down.**
- **Keeping your lower body still on the ground and relaxed, press your upper body off the ground until your elbows are straight and locked out.**
- **Then return to the start position.**



POSITIVE = HAS pain

NEGATIVE = NO pain

Tips to Run the Quick Screen

To run an efficient and effective Quick Screen, use the following tips for testing:

- **Follow the script.** Use the standardized verbal instructions for every test to ensure consistency and reliability across screens.
- **Prioritize pain and understanding.** Always confirm the individual has no pain with the movement and fully understands the instructions before they begin.
- **Control the se-tup first.** Proper starting position is essential. If the set-up is incorrect, stop the movement, restate the instructions, and reset before continuing.
- **Watch the whole movement.** Observe the body globally rather than focusing on individual joints or segments. Movement quality is best assessed by seeing the entire pattern unfold.
- **Score each repetition.** Mentally score every attempt and record the best repetition as the final score for that test.
- **Stay objective.** Assign scores strictly based on the criteria. Avoid interpreting, diagnosing, or explaining the result during the screen.
- **Do not coach the movement.** Once the test begins, avoid cueing or correcting form beyond set-up corrections. The screen measures capability, not coached performance.
- **Limit feedback during the screen.** Let the individual know upfront that results and explanations will be discussed after the screening is complete.
- **Maintain flow and efficiency.** Move smoothly from test to test to keep the screen brief, focused, and repeatable.
- **Create a relaxed environment.** A calm, supportive atmosphere improves compliance and helps reveal true movement patterns.

Remember:

Perfect movement is not the expectation; establishing a baseline and identifying priorities is the purpose of the screen. Establishing a baseline provides the information needed to make appropriate decisions regarding progression and corrective strategies.

The Quick Screen Score Sheet

We have provided a standardized scoring sheet to assist you in learning how to accurately document scores and to support consistent use in your professional setting. The most important consideration when recording scores is consistency—always follow the established scoring criteria and rules.

Remember, the Quick Screen is intentionally designed to be fast and simple. It is not meant to encourage over analysis of individual screens. When documenting results, we recommend recording the score and moving on. In our experience, excessive notetaking on the score sheet does not provide additional benefit and may detract from the efficiency of the screen.

The Quick Screen scoring sheet includes a Raw Score, Final Score, and Total Score. Each of these scores plays a role in determining appropriate intervention strategies, although their importance may vary depending on your professional setting. For example, a fitness professional working one-on-one may rely more heavily on the Raw Score to prioritize protection strategies and guide corrective exercise selection. In contrast, a strength and conditioning coach working with large groups may place greater emphasis on the Final Score to determine next steps for the group as a whole. The Total Score is most useful when comparing individuals or tracking trends across groups.

The Raw Score represents the individual right- and left-side results of applicable tests and allows for bilateral comparison. The Final Score is the lower of the two Raw Scores. For tests that do not assess right and left sides, only a single score is recorded. The Total Score is the sum of all Final Scores.

As clearly outlined in the scoring criteria, any movement that provokes pain receives a score of 0, and referral to an appropriate medical professional is recommended.

Clearing screens are not scored using the numerical scale. Instead, they are used to determine whether pain is present and are documented as Positive (pain) or Negative (no pain).

For the Cervical Flexion and Cervical Rotation tests, minimum range-of-motion criteria are also included. These are recorded as Pass when the criteria are met or Fail when they are not.

These guidelines are intended to help you better understand the scoring system and use the results more effectively. While the Quick Screen is built on a simple grading structure, increased proficiency with the screen and its outcomes will allow for more effective and confident application of corrective strategies.



FMS QUICK SCREEN SCORE SHEET

NAME:

DATE:

TEST		RAW SCORE	FINAL SCORE	COMMENTS
CERVICAL FLEXION				
CERVICAL ROTATION	L			
	R			
CERVICAL EXTENSION CLEARING	+/-			
	+/-			
TOE TOUCH	L			
	R			
SHOULDER MOBILITY	L			
	R			
SHOULDER CLEARING TEST	+/-			
	+/-			
ROTATION	L			
	R			
BALANCE	L			
	R			
SQUAT				
EXTENSION CLEARING TEST	+/-			
TOTAL SCREEN SCORE				

Raw Score: This score is used to denote right and left side scoring.

Final Score: This score is used to denote the overall score for the test. The lowest score for the raw score (each side) is carried over to give a final score for the test. A person who scores a three on the right and a two on the left would receive a final score of two. The final score is then summarized and used as a total score.

Clearing Test: A positive indicates pain. A negative indicates no pain. If pain is present (+) the score for that test would result in a 0.



Corrective Strategy Algorithm

The Quick Screen uses a structured algorithm to guide corrective decisions once the screen is complete. Simply stated, an algorithm is a defined procedure used to guide decision-making, and in this system it directs attention to the primary movement limitation—or "weak link"—identified by the screen results.

The goal is not to correct every limitation at once. Instead, the algorithm helps narrow focus to the single most important priority that is most likely influencing overall movement quality. By addressing this primary factor first, secondary compensations often improve without being directly targeted.

Although the process may appear complex at first, applying the algorithm is quick and efficient. With practice, it takes only a moment to review the score sheet, identify the priority, and determine the appropriate corrective focus.

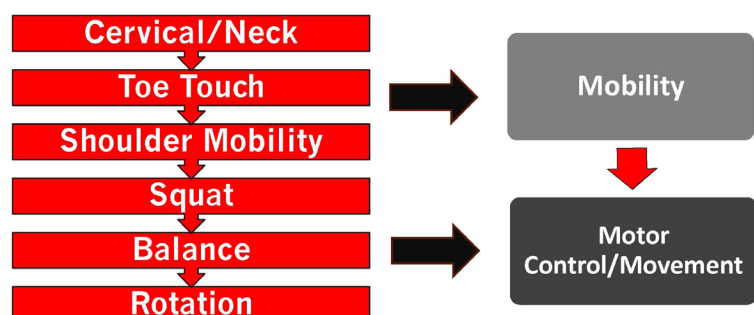
GENERAL SCORING GUIDELINES

The Quick Screen scoring system is designed to provide a clear, objective snapshot of movement quality while guiding appropriate next steps. Scores are assigned based on observable movement criteria, pain response, and pattern integrity rather than interpretation or diagnosis. The guidelines that follow outline how scores should be applied, prioritized, and used to inform corrective decision-making, ensuring consistency, safety, and clarity throughout the screening process.

Some general Scoring guidelines are:

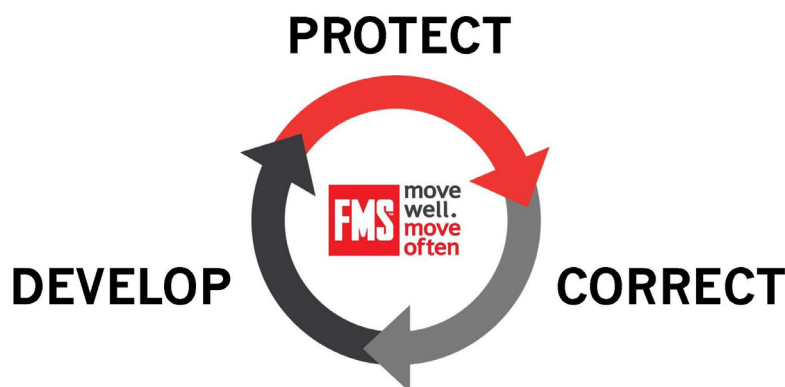
- **A score of zero indicates pain and should be referred to an appropriate medical professional.** Establish a trusted referral network that includes qualified healthcare providers, and whenever possible, identify an SFMA-trained clinician to further evaluate individuals who score a zero.
- **A score 1 means the person is either unable to achieve the required position or unable to perform the movement as instructed, reflecting significant compensation and unacceptable movement quality.** These patterns become a priority in our corrective algorithm and areas of opportunity to influence positive change on movement quality.
- **Remember, scores of 3 are optimal, but scores of 2 are acceptable!** These scores mean that the person moves well in these patterns and are not our immediate concern.
- **Mobility patterns are addressed first, as effective stability and motor control cannot exist in the presence of mobility restrictions.** Restoring appropriate mobility ensures accurate sensory input, which is necessary for the effective development and maximization of stability and motor control
- **Following the order of the algorithm, look for scores of 1 or an asymmetry in order to identify the "weak link".** (Remember the order of the score sheet is designed for efficiently administering the screen. The order of the algorithm is different based on the priorities for the corrective process.)

CORRECTIVE HIERARCHY



Protect, Correct, and Develop

"Protect, Correct, and Develop" represents the ethical foundation for applying movement screening and assessment results. Rooted in the Hippocratic principle of first, do no harm, this approach guides professionals to make informed decisions that prioritize the safety, long-term health, and independence of those in their care. Rather than reacting to movement limitations by simply adding more exercise, this principle emphasizes thoughtful progression based on movement quality.



Protection is always the first priority. Based on screening and assessment findings, this may involve temporarily removing or modifying exercises, activities, or lifestyle demands that reinforce pain or dysfunctional movement patterns. Protection can also include reducing load, volume, or frequency, addressing environmental factors such as work or daily habits, or referring to an appropriate healthcare professional when pain is present. In many cases, simply removing exposure to problematic patterns allows movement quality to improve without the immediate need for corrective interventions.

Once protection is established, correction becomes the focus. Using objective feedback from the screen, targeted corrective strategies are applied to address identified limitations in mobility, stability, or motor control. These strategies are designed to improve the movement baseline and restore efficient, pain-free patterns rather than chasing isolated strength or fitness gains.

Development is the final phase and only occurs after minimum movement standards are met. At this stage, stress, load, and complexity are gradually reintroduced to challenge the improved movement patterns and build capacity, fitness, and performance. By following the sequence of protect, correct, and develop, functional movement professionals avoid building fitness on dysfunction and instead ensure that increased demands are supported by quality movement, leading to more sustainable progress and long-term success.

Corrective Strategy Example: Cervical Patterns

After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

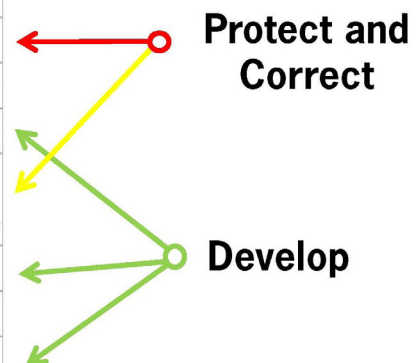
In this example, cervical rotation is the focus, indicated by a failing score. As a result, the cervical spine becomes the top priority within the corrective hierarchy.

The Protect and Correct designation highlights movement patterns that require immediate attention. Protecting the cervical spine means temporarily removing or modifying exercises and activities that place excessive stress on the neck, such as high-impact, high-velocity, or power-based movements. Corrective strategies are then applied to address the identified limitation before additional stress is introduced.

The yellow indicator draws attention to shoulder mobility. Although shoulder mobility scores are acceptable, this area is flagged for awareness rather than direct intervention. This reflects the close relationship between the cervical spine and the shoulders—limitations or dysfunction in one region may influence the other. Improvements in cervical function may positively affect shoulder mobility, which is why this pattern should be monitored during reassessment, particularly when progressing overhead activities.

The Develop designation identifies movement patterns that score well and can be safely trained. In this example, lower-body patterns such as toe touch, rotation, balance, and squat demonstrate adequate movement quality. These patterns can be stressed and developed to maintain overall training intensity while corrective efforts focus on the cervical spine.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L FAIL	FAIL
	R PASS	
CERVICAL EXTENSION CLEARING(+/-)	L -	-
	R -	
TOE TOUCH	L 3	3
	R 3	
SHOULDER MOBILITY	L 2	2
	R 3	
SHOULDER CLEARING TEST(+/-)	L -	-
	R -	
ROTATION	L 3	3
	R 3	
BALANCE	L 3	2
	R 2	
SQUAT	3	3
EXTENSION CLEARING TEST	+/- -	
TOTAL SCREEN SCORE		13



EXERCISE EXAMPLES FOR CERVICAL PATTERNS:

These exercises can be found in the exercise library at www.functionalmovement.com

- Breathing exercises (Crocodile Breathing, 90/90 Breathing)
- Assisted Supine T-Bar Cervical Bobbleheads
- Rolling Upper Body
- Quadruped T-Spine Rotation Lumbar Locked/ Quadruped T-Spine Rotation
- Assisted Standing T-Bar Cervical Rotation
- Assisted Standing T-Bar Cervical Rotation

Corrective Strategy Example: Toe Touch

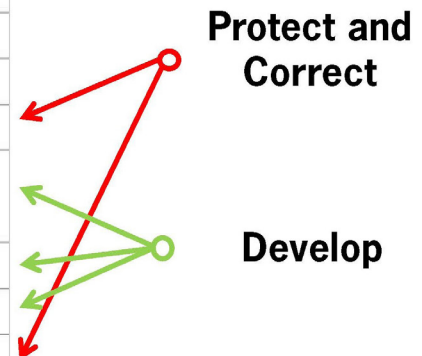
After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

In this example, the toe touch is the primary pattern of focus. The score of 1 indicates the individual is unable to perform the movement without significant compensation. The squat pattern also scores a 1, which is not unexpected, as both movements rely on the ability to hinge effectively at the hips.

Although both patterns are dysfunctional, the toe touch is prioritized because it is the simpler and more fundamental movement pattern. An effective squat cannot be achieved without first restoring the ability to hinge, which is assessed through the toe touch.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L PASS R PASS	PASS
CERVICAL EXTENSION CLEARING (+/-)	L - R -	-
TOE TOUCH	L 1 R 1	1
SHOULDER MOBILITY	L 3 R 3	3
SHOULDER CLEARING TEST (+/-)	L - R -	-
ROTATION	L 2 R 2	2
BALANCE	L 3 R 2	2
SQUAT	1	1
EXTENSION CLEARING TEST	+/- -	-
TOTAL SCREEN SCORE		9



The Protect and Correct designation reflects the need to temporarily remove or modify activities that heavily load the hip hinge. With a toe touch score of 1, movements such as squatting and loaded hinge-based exercises, including deadlifting, should be avoided. These activities would place excessive stress on a dysfunctional pattern and increase the likelihood of compensation.

The Develop designation identifies movement patterns that score 2 or 3 and can continue to be trained. In this case, shoulder mobility scores a 3, while rotation and balance score 2, indicating these patterns are acceptable and can be safely stressed. Training these areas allows the individual to maintain overall strength and conditioning while corrective strategies focus on improving the toe touch.

EXERCISE EXAMPLES FOR THE TOE TOUCH PATTERN:

These exercises can be found in the exercise library at www.functionalmovement.com

- Assisted Single Leg Lowering
- Leg Raise Core Engagement with FMT
- Straight Leg Bridge
- Leg Lock Bridge
- Half Kneeling Rotation with Dowel
- Hip Flexor Stretch with Core Activation
- Split Stance Toe Touch Progression

Corrective Strategy Example: Shoulder Mobility

After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

In this example, shoulder mobility becomes the primary focus within the corrective hierarchy. The score sheet shows a shoulder mobility score of 1, indicating significant limitation and compensation in this pattern. As a result, shoulder mobility is categorized under Protect and Correct.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L PASS R PASS	PASS
CERVICAL EXTENSION CLEARING (+/-)	L - R -	-
TOE TOUCH	L 3 R 3	3
SHOULDER MOBILITY	L 1 R 1 SHOULDER CLEARING TEST (+/-) -	1
ROTATION	L 2 R 2	2
BALANCE	L 3 R 3	3
SQUAT	2	2
EXTENSION CLEARING TEST +/-	-	-
TOTAL SCREEN SCORE		11

Protect and Correct

Develop

Protecting the shoulder pattern means temporarily limiting upper-body activities that place high stress on the shoulders, particularly those that require significant mobility, loading, or overhead positioning. Rather than removing training entirely, the focus shifts to maintaining intensity through patterns that score well. In this case, toe touch scores a 3, while rotation, balance, and squat score 2 or higher, indicating that lower-body training can continue without restriction. Exercises such as two-legged deadlifting, squatting, and lower-body strength work can be emphasized to maintain conditioning while minimizing upper-body stress.

Although two-legged deadlifting involves the upper body, it can be appropriate in this case because it encourages spinal stability and postural control without demanding excessive shoulder mobility. This allows the individual to continue training effectively while protecting the dysfunctional shoulder pattern.

EXERCISE EXAMPLES FOR SHOULDER MOBILITY:

These exercises can be found in the exercise library at www.functionalmovement.com

- Breathing exercises (Crocodile Breathing, 90/90 Breathing)
- T-Spine Rotation with Rib Grab
- T-Spine Rotation with Reach
- Trunk Stability Rotation Knees Flexed
- Quadruped T-Spine Rotation
- Half Kneeling KB Halo
- Single Arm Toe Touch Progression

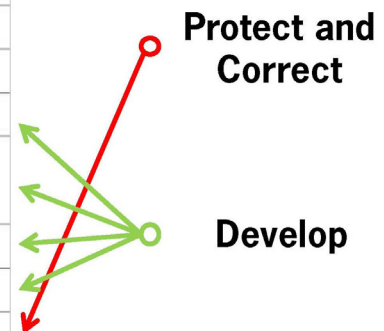
Corrective Strategy Example: Squat

After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

In this example, the squat pattern becomes the primary focus within the corrective hierarchy. The score sheet shows a squat score of 1, indicating significant limitation and compensation. Despite this, several prerequisite patterns score acceptably, including toe touch (2), rotation (2), and balance (2). These results help narrow the likely contributors to the dysfunctional squat.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L PASS	PASS
	R PASS	
CERVICAL EXTENSION CLEARING (+/-)	L -	-
	R -	
TOE TOUCH	L 2	2
	R 2	
SHOULDER MOBILITY	L 3	3
	R 3	
SHOULDER CLEARING TEST (+/-)	L -	-
	R -	
ROTATION	L 2	2
	R 2	
BALANCE	L 2	2
	R 2	
SQUAT	1	1
EXTENSION CLEARING TEST	+/- -	-
TOTAL SCREEN SCORE		10



Because the toe touch score is acceptable, the individual demonstrates an effective hip hinge. This allows deadlifting to remain in the program, as it reinforces posterior-chain strength without placing excessive demand on the squat pattern or ankle range of motion. Deadlifting is a total-body exercise that can be safely emphasized in this case while the squat is being corrected.

The Protect and Correct designation applies specifically to the squat pattern. Squatting and squat-based loading strategies should be temporarily removed from training. Given the combination of a squat score of 1 with otherwise acceptable movement patterns, limited ankle mobility is considered a likely underlying factor. Until ankle mobility is assessed and improved, higher-level activities that place significant demands on ankle range of motion—such as plyometric exercises—should also be avoided.

The Develop designation identifies movement patterns that can continue to be trained. Because balance, rotation, and toe touch are acceptable, lower-body activities such as step-ups, split squats, and controlled single-leg work may still be appropriate, provided they do not excessively stress the ankle or reinforce poor squat mechanics.

EXERCISE EXAMPLES FOR SQUAT:

These exercises can be found in the exercise library at www.functionalmovement.com

- Dorsiflexion from Half Kneeling with Dowel
- Open Half Kneeling Ankle Mobility with KB
- Brettzel 3.0
- Tall Kneeling Turns
- Toe Touch Squat

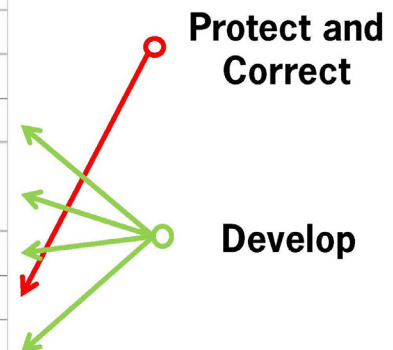
Corrective Strategy Example: Balance

After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

In this example, balance becomes the primary focus within the corrective hierarchy. The score sheet shows a balance score of 1, indicating a significant deficit in stability and motor control. All preceding mobility-based patterns—including cervical movement, toe touch (2), shoulder mobility (2), and squat (2)—score acceptably, confirming that mobility is not the limiting factor. This positions balance as the next logical priority.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L PASS	PASS
	R PASS	
CERVICAL EXTENSION CLEARING (+/-)	L -	-
	R -	
TOE TOUCH	L 2	2
	R 2	
SHOULDER MOBILITY	L 2	2
	R 2	
SHOULDER CLEARING TEST (+/-)	L -	-
	R -	
ROTATION	L 2	2
	R 3	
BALANCE	L 1	1
	R 1	
SQUAT	L 2	2
	R 2	
EXTENSION CLEARING TEST	+/- -	-
TOTAL SCREEN SCORE		9



Because balance reflects the integration of mobility, stability, and motor control, corrective strategies now shift toward improving coordination and control rather than restoring range of motion. The Protect and Correct designation applies specifically to high-level and dynamic single-leg activities. Exercises that place excessive demand on balance—such as loaded single-leg work, high-velocity movements, plyometrics, and even running—should be temporarily limited, as they may reinforce compensations or increase risk.

Despite the balance limitation, several patterns can continue to be trained under the Develop designation. The individual demonstrates acceptable movement quality in squatting, toe touch, rotation, and upper-body patterns. As a result, more stable, bilateral strength exercises such as squatting and deadlifting can remain in the program, allowing the lower body to be trained without excessive balance demands.

Upper-body movement and rotational patterns also score well and can be safely developed to maintain overall training capacity. This approach allows the practitioner to continue building strength and conditioning while corrective strategies focus on improving balance in a controlled and progressive manner.

EXERCISE EXAMPLES FOR BALANCE:

These exercises can be found in the exercise library at www.functionalmovement.com

- Rolling Patterns
- Quadruped to Half Kneeling
- Half Kneeling KB Halo
- Half Kneeling Chops
- Stride With Torso Rotation
- Toe Touch Progression

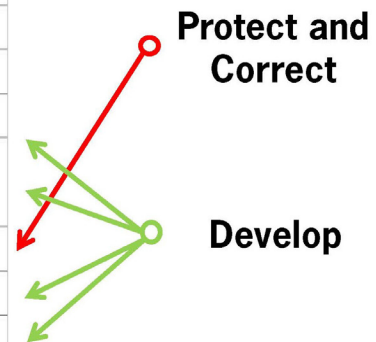
Corrective Strategy Example: Rotation

After completing the Quick Screen, the score sheet is used to guide programming decisions. Always follow the scoring rules by identifying pain first, then using scores of 1 or Fail to establish corrective priorities. Once priorities are identified, apply the corrective hierarchy, beginning with the mobility-based patterns—cervical movement, toe touch, and shoulder mobility—before progressing to stability and motor control strategies.

This is an example of how the Quick Screen score sheet is used to guide decision-making through the Protect, Correct, and Develop framework.

In this example, rotation is the final pattern addressed within the corrective hierarchy. The score sheet shows a rotation score of 1, indicating a significant deficit in this movement pattern. All prerequisite patterns—including cervical movement, toe touch (3), shoulder mobility (3), balance (2), and squat (3)—score acceptably, confirming that fundamental mobility and basic stability are not limiting factors.

TEST	RAW SCORE	FINAL SCORE
CERVICAL FLEXION	PASS	PASS
CERVICAL ROTATION	L PASS	PASS
	R PASS	
CERVICAL EXTENSION CLEARING(++)	L -	-
	R -	
TOE TOUCH	L 3	3
	R 3	
SHOULDER MOBILITY	L 3	3
	R 3	
SHOULDER CLEARING TEST(++)	L -	-
	R -	
ROTATION	L 1	1
	R 1	
BALANCE	L 2	2
	R 2	
SQUAT	L 3	3
	R 3	
EXTENSION CLEARING TEST	+/+	-
TOTAL SCREEN SCORE		12



Rotation is a highly complex pattern that requires the integration of mobility, stability, and motor control across multiple regions of the body. Because all foundational patterns score well, a rotation score of 1 suggests the issue is not a lack of range of motion, but rather a motor control and coordination deficit. This is one of the primary reasons rotation is addressed last in the hierarchy—effective rotation cannot exist without adequate foundational movement quality.

The Protect and Correct designation applies specifically to rotational loading. Activities that place high demands on rotation—such as rotational power exercises, dynamic twisting movements, and high-velocity rotational drills—should be temporarily removed. In complex patterns like rotation, compensations are easy to hide, which can mask underlying deficits and increase injury risk over time.

Despite the limitation in rotation, the Develop designation highlights several patterns that can continue to be trained. Because lower-body, upper-body, and linear movement patterns score well, strength training can continue using single-plane, non-rotational exercises. This allows the individual to maintain training intensity while avoiding movements that excessively stress the dysfunctional rotational pattern.

EXERCISE EXAMPLES FOR ROTATION:

These exercises can be found in the exercise library at www.functionalmovement.com

- Rolling Patterns
- Hard Roll
- Brettzel 2.0
- Tall Kneeling Turns
- Stride With Torso Rotation

Appendix 1

Verbal Instructions

Verbal Instructions

The following statement begins the screen and applies to all tests.

Please let me know if there is any pain during the following movements. And if at any time you do not understand the instructions, stop me for clarification. We are going to perform each movement one at a time and in a smooth and controlled motion. Please wait for me to confirm that you have the proper set-up position and then I will signal you to begin the movement.

For consistency throughout all screens, this script can be used. The bold words below should be repeated to the client.

CERVICAL FLEXION CLEARING

- Stand tall with your feet together, arms at your sides, and mouth closed.
- Place two fingers stacked one on top of the other, at the top of your chest bone.
- While keeping your mouth closed, slowly bring your chin down to your fingers.
- Hold the bottom position for a count of one and then return to the starting position.

CERVICAL ROTATION AND EXTENSION CLEARING

- Stand tall with your feet together, arms at your sides, and mouth closed.
- While keeping your mouth closed, slowly rotate your head to the right as far as possible.
- Hold the position for a count of one.
- Now lower your right ear towards your right shoulder.
- Return to the start position.
- Repeat the same process by turning your head to the left as far as possible, then lowering your ear towards your left shoulder.

TOE TOUCH

- Start by standing with your feet together and your hands by your sides.
- Step forward with your right foot so that the inside of your right heel is even with end of your left big toe.
- Now, place your hands on top of each other.
- Keeping your knees straight but not locked out, look down and bend forward to try to touch the toes of your left foot.
- Return to the upright position.

If after 3 attempts they are NOT able to touch the toes of the left foot, continue to the next part of the test:

- Keeping your knees straight but not locked out, look down and bend forward again, this time trying to touch the toes of your right foot, and then return to your upright position.
- Switch foot positions and repeat on the opposite side.

SHOULDER MOBILITY

- Stand tall with your feet together and arms hanging comfortably.
- Make a fist so your fingers are around your thumbs.
- In one motion, reach the right fist over the head and down your back as far as possible while simultaneously reaching your left fist up your back as far as possible.
- Do not "creep" your hands closer after the initial placement.
- Repeat with the left fist reaching over the head while the right fist reaches up your back.

SHOULDER CLEARING

- While standing tall, reach your right arm over your head to touch the top of your left shoulder blade.
- Now, reach your right hand behind your back to try to touch the bottom of your left shoulder blade.
- Repeat both patterns with your left arm.

ROTATION

- Start by standing with your feet together and your hands by your sides.
- Step forward with your right foot so that the inside of your right heel is even with the end of your left big toe.
- Extend your arms directly in front of your chest with your fingers interlocked and thumbs up and in line with your nose.
- Without moving your foot position, rotate your body to the right as far as possible.
- Return to the start position.

If after 3 attempts they are NOT able to rotate beyond 90 degrees, continue to the next part of the test:

- Move your foot back so that your feet are together, now with your arms directly in front of your chest with your fingers interlocked and thumbs up and in line with your nose.
- Without moving your foot position, rotate your body to the right as far as possible.
- Return to the start position.
- Repeat the entire process starting with left foot forward rotating to the left.

BALANCE

- Start by standing with your feet together and your hands by your sides.
- Lift your knee and thigh up to be in line with your waist.
- Hold this position, keeping your eyes open.

If they ARE able to maintain balance for 10 seconds with eyes open, proceed to the next part of the test:

- Now close your eyes and maintain this position.

SQUAT

- Start by standing with your feet together and extend your arms directly in front of your chest with your hands in a fist position.
- Now, squat down as far as you can, keeping your heels on the ground and chest tall.
- At the bottom squat position, lower your fists to your sides, trying to touch them on the ground alongside your feet.
- Then come back up to the starting position.

If they are NOT able to put their fist to the ground, continue to the next part of the test:

- Start by standing with your feet together and extend your arms directly in front of your chest with your thumbs pointing up and fingers straight out.
- Now, squat down as far as you can, keeping your heels on the ground and chest tall.
- At the bottom squat position, lower your fingers to your sides, trying to touch them on the ground alongside your feet.
- Then come back up to the starting position.

SPINE EXTENSION CLEARING

- Lie on your stomach, with your hands underneath your shoulders and your palms facing down.
- Keeping your lower body still on the ground and relaxed, press your upper body off the ground until your elbows are straight and locked out.
- Then return to the start position.

Appendix 2

Quick Screen Score Sheets

FMS QUICK SCREEN SCORE SHEET

NAME: _____

DATE: _____

TEST		RAW SCORE	FINAL SCORE	COMMENTS
CERVICAL FLEXION (Pass/Fail)				
FLEXION CLEARING TEST	+/-			
CERVICAL ROTATION (Pass/Fail)	L			
	R			
CERVICAL EXTENSION CLEARING (+/-)	L			
	R			
TOE TOUCH	L			
	R			
SHOULDER MOBILITY	L			
	R			
SHOULDER CLEARING TEST (+/-)	L			
	R			
ROTATION	L			
	R			
BALANCE	L			
	R			
SQUAT				
EXTENSION CLEARING TEST	+/-			
TOTAL SCREEN SCORE				

Raw Score: This score is used to denote right and left side scoring.

Final Score: This score is used to denote the overall score for the test. The lowest score for the raw score (each side) is carried over to give a final score for the test. A person who scores a three on the right and a two on the left would receive a final score of two. The final score is then summarized and used as a total score.

Clearing Test: A positive indicates pain. A negative indicates no pain. If pain is present (+), the score for that test would result in a 0.

FMS QUICK SCREEN SCORE SHEET

NAME: _____

DATE: _____

TEST		RAW SCORE	FINAL SCORE	COMMENTS
CERVICAL FLEXION (Pass/Fail)				
FLEXION CLEARING TEST	+/-			
CERVICAL ROTATION (Pass/Fail)	L			
	R			
CERVICAL EXTENSION CLEARING (+/-)	L			
	R			
TOE TOUCH	L			
	R			
SHOULDER MOBILITY	L			
	R			
SHOULDER CLEARING TEST (+/-)	L			
	R			
ROTATION	L			
	R			
BALANCE	L			
	R			
SQUAT				
EXTENSION CLEARING TEST	+/-			
TOTAL SCREEN SCORE				

Raw Score: This score is used to denote right and left side scoring.

Final Score: This score is used to denote the overall score for the test. The lowest score for the raw score (each side) is carried over to give a final score for the test. A person who scores a three on the right and a two on the left would receive a final score of two. The final score is then summarized and used as a total score.

Clearing Test: A positive indicates pain. A negative indicates no pain. If pain is present (+), the score for that test would result in a 0.

FMS QUICK SCREEN SCORE SHEET

NAME: _____

DATE: _____

TEST		RAW SCORE	FINAL SCORE	COMMENTS
CERVICAL FLEXION (Pass/Fail)				
FLEXION CLEARING TEST	+/-			
CERVICAL ROTATION (Pass/Fail)	L			
	R			
CERVICAL EXTENSION CLEARING (+/-)	L			
	R			
TOE TOUCH	L			
	R			
SHOULDER MOBILITY	L			
	R			
SHOULDER CLEARING TEST (+/-)	L			
	R			
ROTATION	L			
	R			
BALANCE	L			
	R			
SQUAT				
EXTENSION CLEARING TEST	+/-			
TOTAL SCREEN SCORE				

Raw Score: This score is used to denote right and left side scoring.

Final Score: This score is used to denote the overall score for the test. The lowest score for the raw score (each side) is carried over to give a final score for the test. A person who scores a three on the right and a two on the left would receive a final score of two. The final score is then summarized and used as a total score.

Clearing Test: A positive indicates pain. A negative indicates no pain. If pain is present (+), the score for that test would result in a 0.

FMS QUICK SCREEN SCORE SHEET

NAME: _____

DATE: _____

TEST		RAW SCORE	FINAL SCORE	COMMENTS
CERVICAL FLEXION (Pass/Fail)				
FLEXION CLEARING TEST	+/-			
CERVICAL ROTATION (Pass/Fail)	L			
	R			
CERVICAL EXTENSION CLEARING (+/-)	L			
	R			
TOE TOUCH	L			
	R			
SHOULDER MOBILITY	L			
	R			
SHOULDER CLEARING TEST (+/-)	L			
	R			
ROTATION	L			
	R			
BALANCE	L			
	R			
SQUAT				
EXTENSION CLEARING TEST	+/-			
TOTAL SCREEN SCORE				

Raw Score: This score is used to denote right and left side scoring.

Final Score: This score is used to denote the overall score for the test. The lowest score for the raw score (each side) is carried over to give a final score for the test. A person who scores a three on the right and a two on the left would receive a final score of two. The final score is then summarized and used as a total score.

Clearing Test: A positive indicates pain. A negative indicates no pain. If pain is present (+), the score for that test would result in a 0.